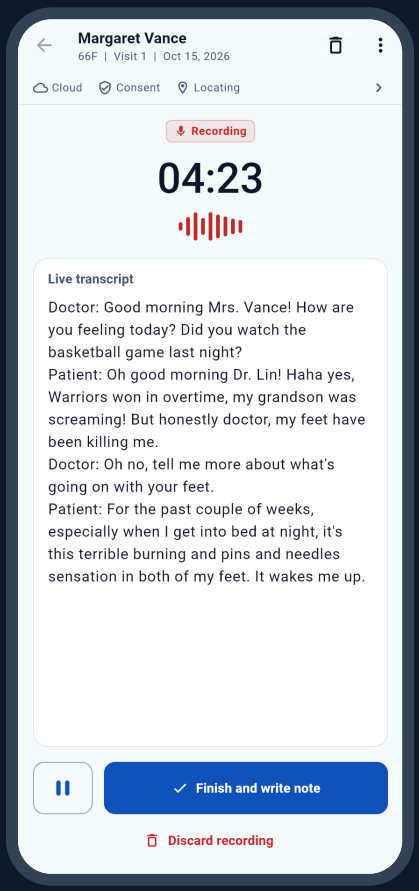
SOLIS
by ML Dynamic

PRODUCT OVERVIEW

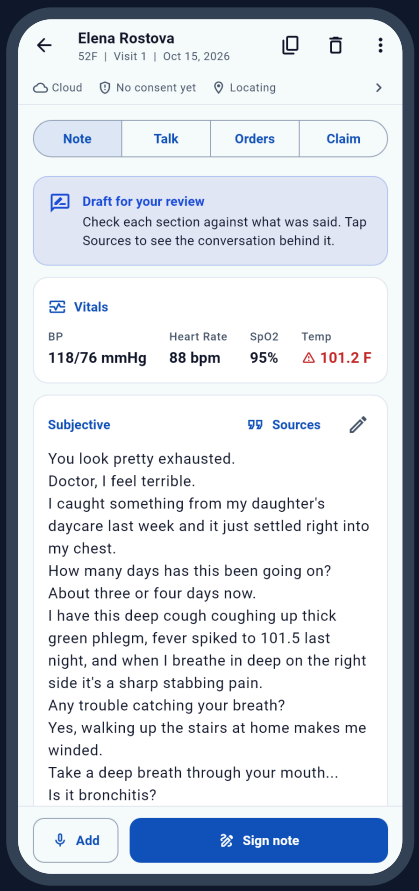
From the exam room conversation to a clean claim, on one phone.

SOLIS listens to the visit with the patient's consent, writes the note, codes the diagnoses from what the patient actually said, checks the claim against the rules payers deny on, and exports it ready for billing.



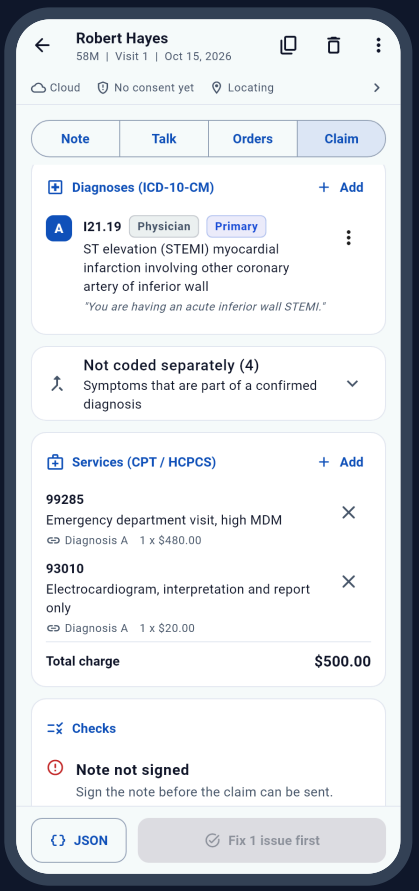
Capture

Record with consent, pause any time.



Review

A draft note to check and sign.



Bill

Coded, checked, exportable.

WHY SOLIS

Documentation and billing are where a visit's time and money leak.

Ambient scribes have shown they can cut note-writing time, but physicians still report that long notes and heavy editing are the main drawback. And a well-documented visit can still be denied if the codes on the claim don't line up.

Notes that need less editing

Each SOAP section is a separate card with one-tap edit, and a Sources button shows the exact words from the visit behind it, so checking takes seconds instead of rereading a transcript.

Coding from the patient's own words

Symptoms are coded from how the patient describes them ("my feet burn at night" becomes paresthesia, R20.2). Diagnoses the physician states with hedged language wait for confirmation, as outpatient coding rules require.

Denials caught before submission

Every procedure must be supported by a related diagnosis. Place of service, sex-specific and age-limited codes, and new versus established patient are checked before the claim is marked ready.

Billing without a hand-off

The claim exports in CMS-1500 and X12 837P structure, so a practice can feed its billing system directly instead of sending charts to an outside billing company and waiting.

Time and revenue impact will be measured in the pilot program and reported per clinic. This document makes no savings claims that have not yet been measured.

Physician experience with ambient scribes: Physician Perspectives on Ambient AI Scribes, JAMA Network Open (jamanetwork.com/journals/jamanetworkopen/fullarticle/2831866). Outpatient uncertain-diagnosis rule: ICD-10-CM Official Guidelines, Section IV.H.

HOW IT WORKS

Five steps, one device.

1

Scan a card

Insurance card, driver's license barcode or SSN card is read on the phone.

2

Open the visit

One account per patient; each date of service is Visit 1, 2, 3.

3

Record

Consent first, then the conversation is captured and split by speaker.

4

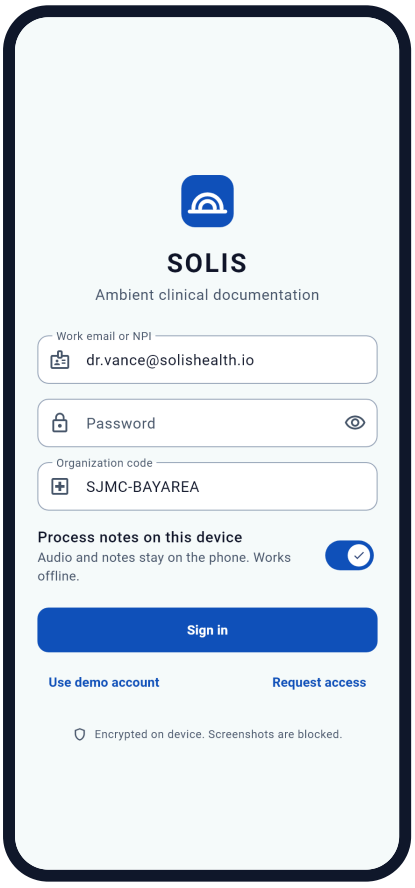
Review and sign

Note, transcript and orders, each traceable to what was said.

5

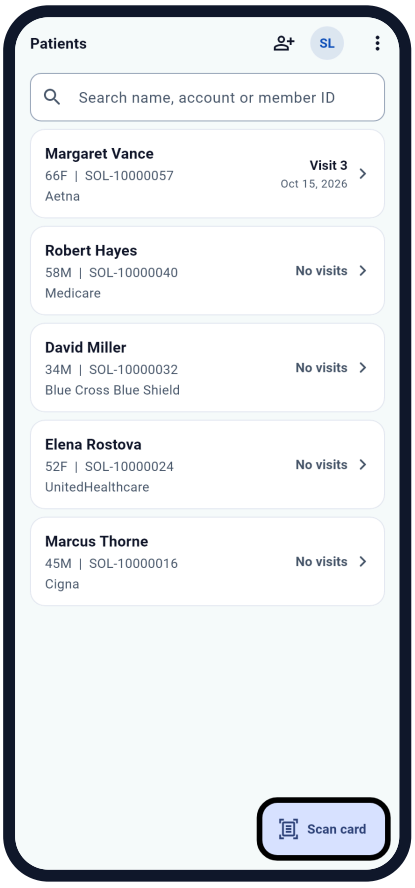
Claim ready

Codes checked against payer rules, exported for billing.



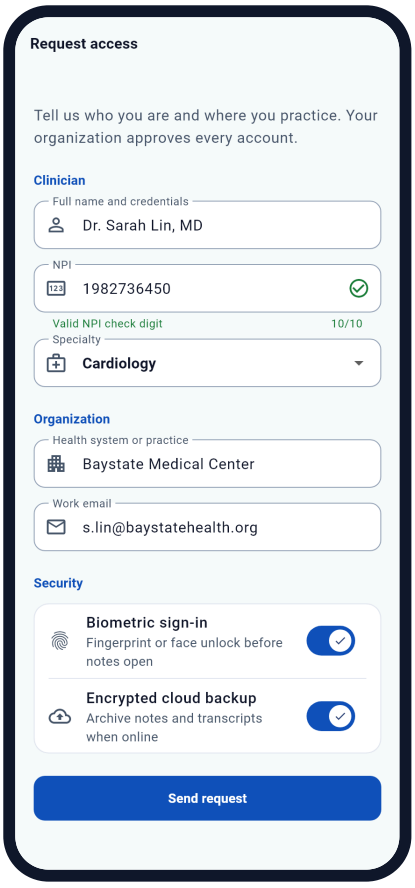
Sign in

Choose whether notes are written on the device or in the cloud.



Patients

Search by name, account number or member ID. Scan card is the main action.



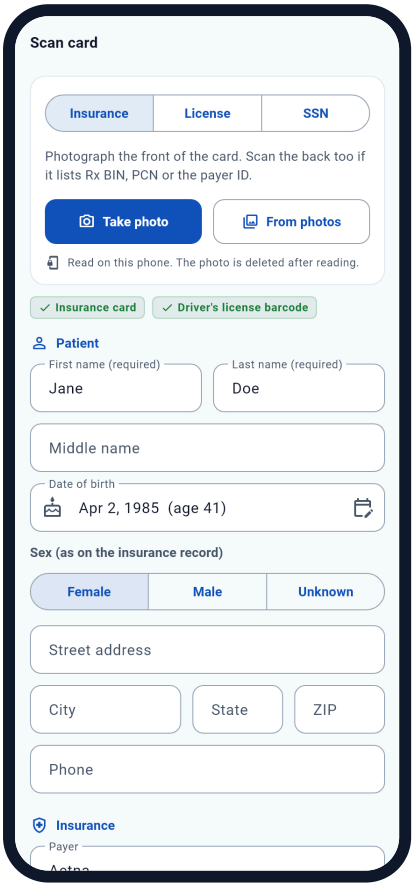
Request access

The NPI is checked with the CMS check-digit rule as it is typed.

STEP 1 AND 2

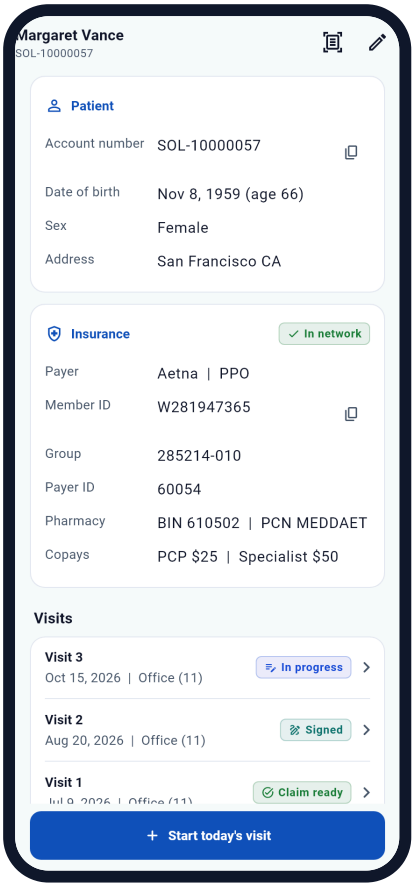
Scan once. One account, every visit under it.

Card text and the license barcode are read on the device and the photo is deleted after reading. Every field stays editable. If the card matches an existing patient, SOLIS offers that account instead of creating a duplicate.



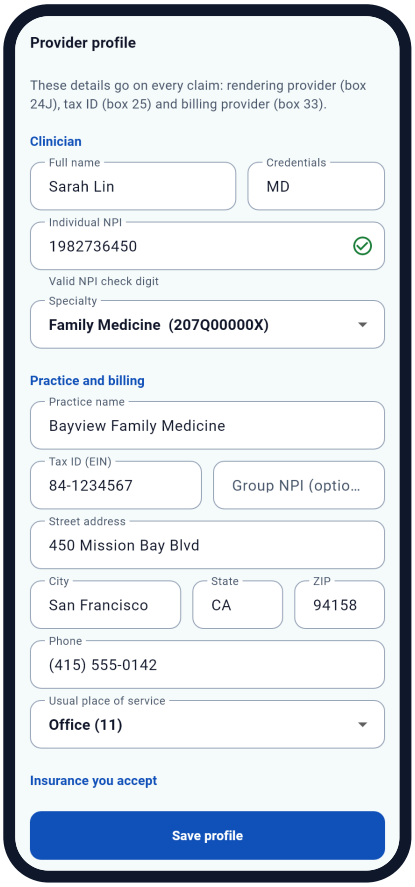
Review what was read

Name, date of birth, sex, payer, member ID, group, Rx BIN and PCN, copays.



Patient account

Account number with a check digit, an in-network check, and numbered visits with status.



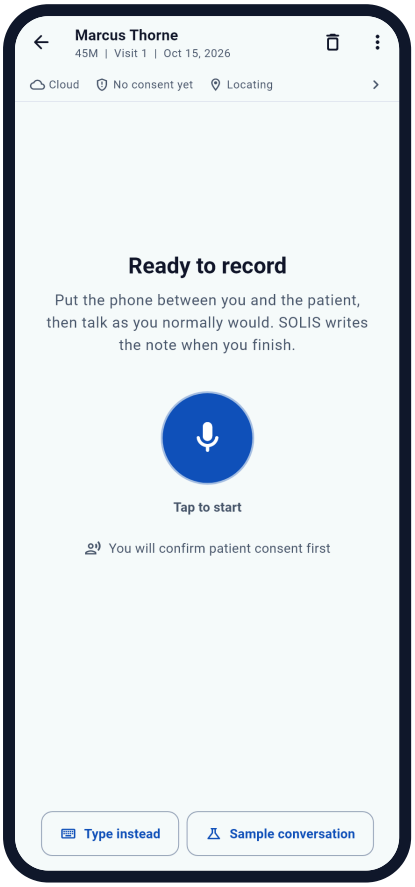
Provider profile

NPI, tax ID, taxonomy, practice address and accepted payers go on every claim.

STEP 3

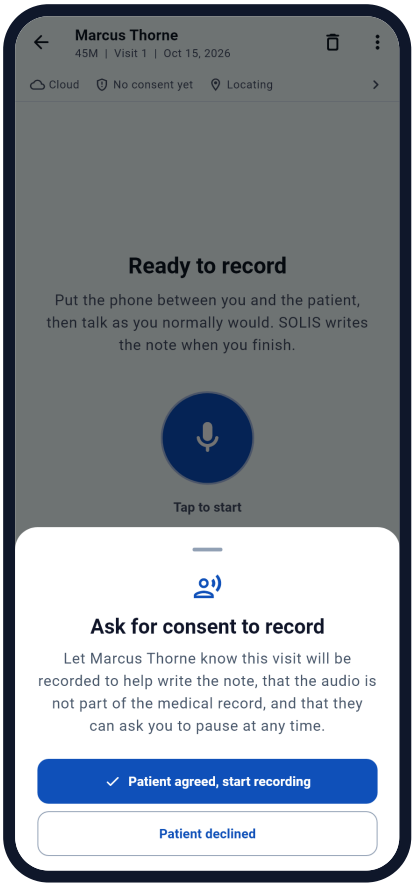
Capture the visit, with consent built in.

One large button starts recording after the physician confirms the patient agreed. The consent time is saved with the note. Recording can pause for a sensitive topic and keeps running with the screen off.



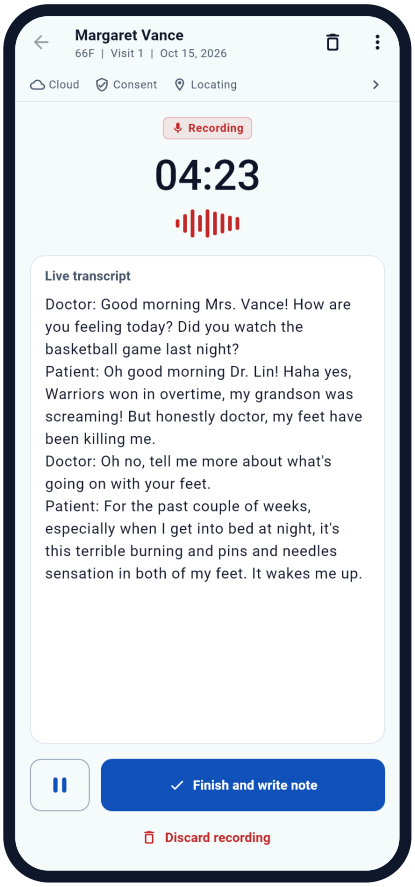
Ready to record

Or type the conversation instead.



Consent first

A short script for the patient, and a clear "declined" path.



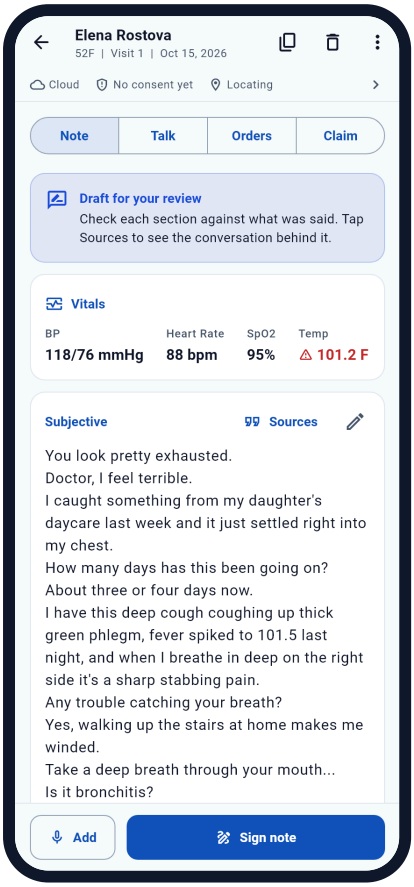
Recording

Timer, live transcript, pause, finish, or discard with nothing kept.

STEP 4

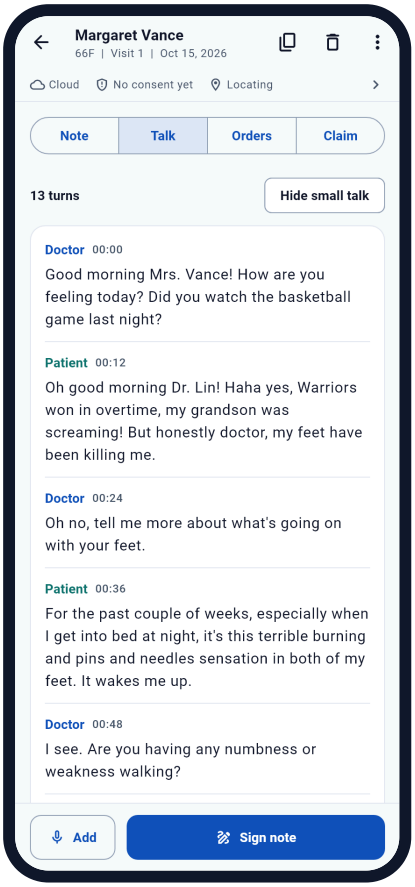
Review the note against what was actually said.

Vitals are recorded only when they are stated in the visit; nothing is filled in with assumed values. The transcript separates doctor and patient, and orders are sorted with STAT items first.



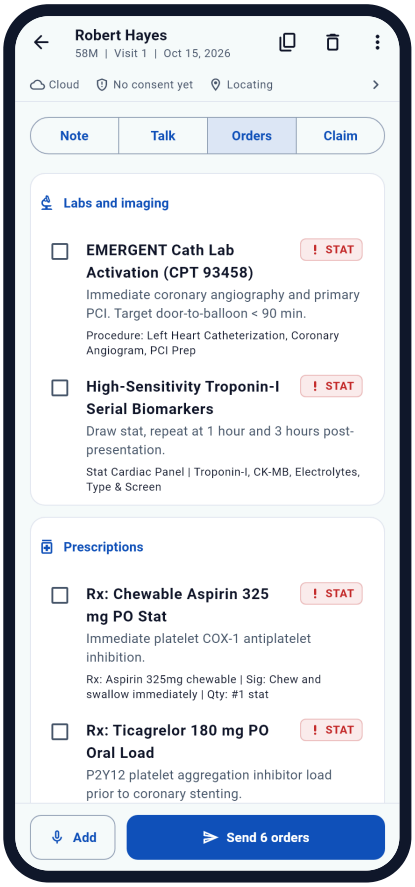
Note

Draft banner, vitals with abnormal values flagged, Sources per section.



Talk

Speaker turns, small talk can be hidden, raw transcript archived.



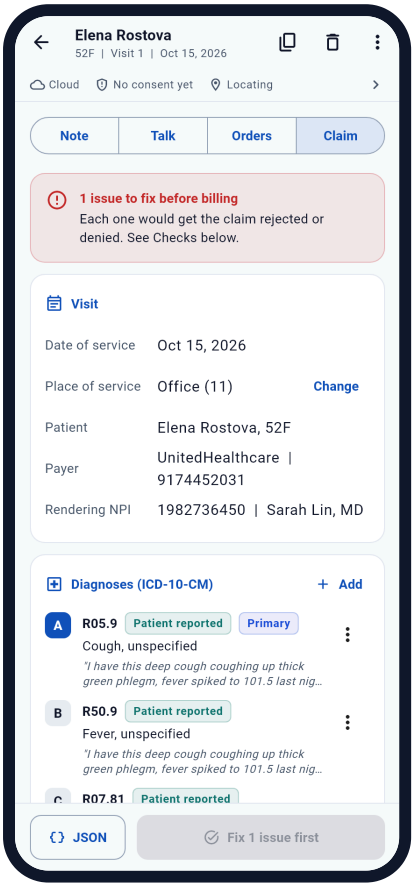
Orders

Labs, imaging, prescriptions and referrals, sent after confirmation.

STEP 5

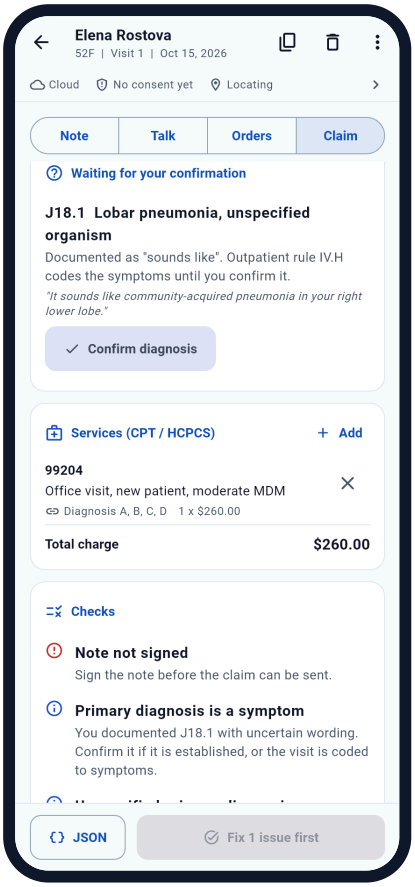
A claim that explains itself.

Diagnoses appear in claim order with pointer letters, each with the words that support it. Uncertain diagnoses wait for confirmation. Symptoms that belong to a confirmed diagnosis are folded in. Every service line shows which diagnosis supports it.



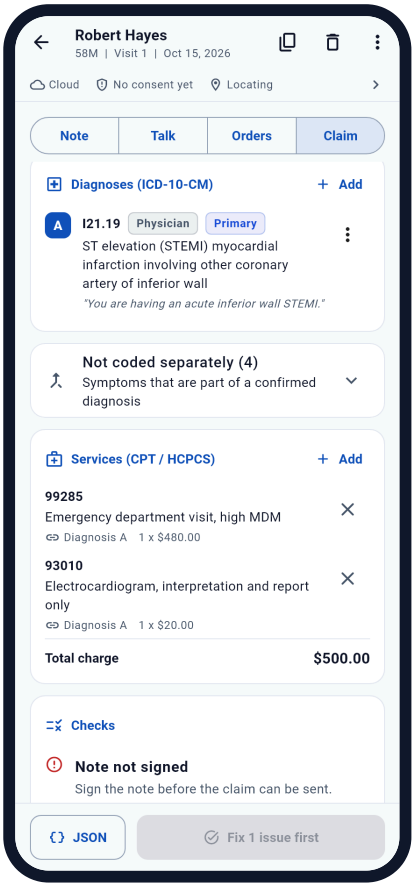
Coded from the patient

"Sounds like pneumonia" is held; cough and fever are coded.



Confirm and check

One tap to confirm. Checks explain anything a payer would reject.



Setting-aware

An ER visit bills the ER code, and the EKG as interpretation only.

CODING RULES

The checks that keep a claim from being denied.

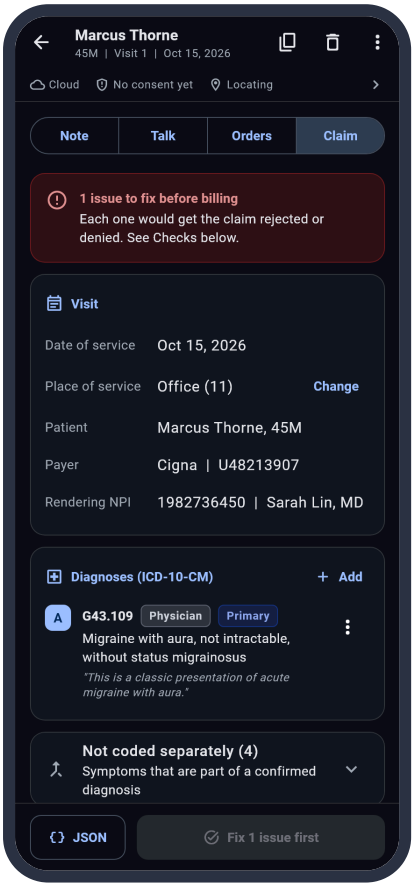
Rule	What SOLIS does
Patient language first	Symptoms are coded from the patient's own words, in the order the patient raised them, so the chief complaint leads.
Uncertain diagnoses	"Sounds like", "suspicious for", "consistent with" and similar wording are not coded until the physician confirms (ICD-10-CM outpatient guideline IV.H).
Integral symptoms	Symptoms that are part of a confirmed diagnosis, such as chest pain with a heart attack, are not coded separately.
Diagnosis supports procedure	Each CPT code must link to a diagnosis in a related body system. A headache cannot support an upper GI endoscopy.
Place of service	The visit code follows the setting: office, telehealth, emergency room, hospital, nursing facility, home. Ambulance bills transport codes, never a visit code.
Sex and age	Sex-specific codes (pregnancy, prostate) and age-limited codes (newborn, preventive age bands) are checked against the patient.
New or established	Derived from the patient's visit history; a new-patient code on a return visit is flagged.
Performed vs ordered	Tests only ordered elsewhere are not billed by the practice. An EKG read in a facility bills interpretation only.
Claim export	CMS-1500 boxes and X12 837P loops in one JSON document, with diagnosis pointers filled in.

Place of service codes: CMS Place of Service Code Set. Sex and age edits modeled on the CMS Medicare Code Editor definitions; commercial payer edits may differ.

DESIGN

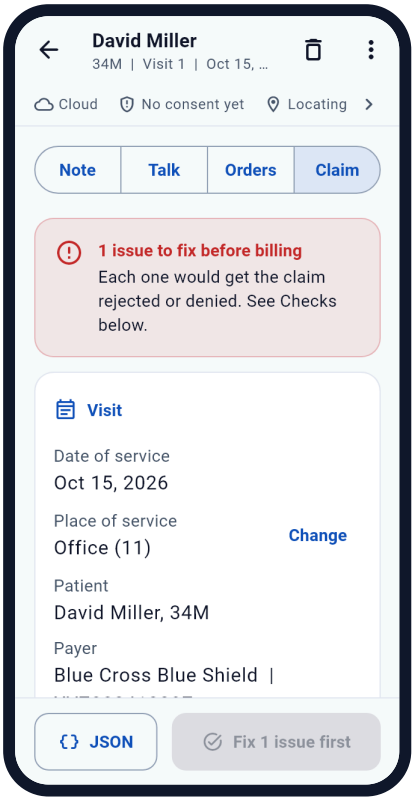
Built for exam-room conditions.

Body text is never below 14 points and the note reads at 16. Touch targets are at least 48 dp. Status always pairs an icon with a word, never color alone. Light and dark themes, and layouts that hold on small phones.



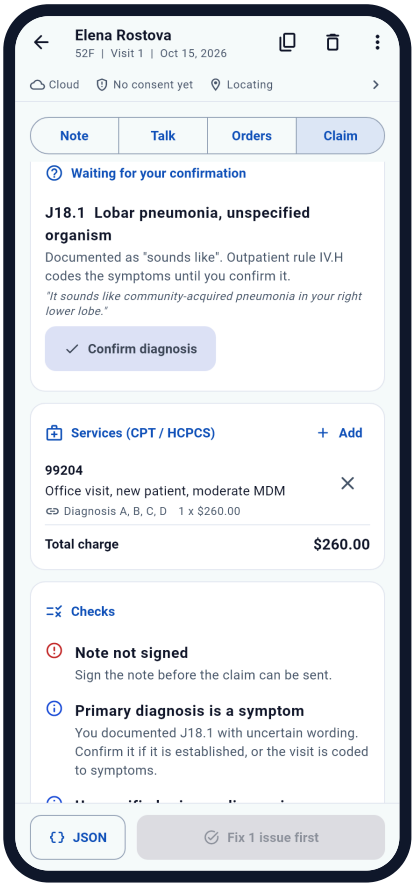
Dark theme

For night shifts and dim rooms.



Small phones

Rows stack instead of squeezing on 320 dp screens.



Plain language

Every check says what is wrong and how to fix it.

PRIVACY AND STATUS

What is in place today, and what comes next.

Area	Status
Card scanning	In place Read on the device; the photo is deleted after reading. Only the last 4 digits of an SSN are kept.
Screen protection	In place Screenshots, screen recording and recent-app previews are blocked.
Consent and audit trail	In place Consent time, UTC timestamp and location are saved with each visit.
On-device notes	In place Notes can be written entirely on the phone, without sending audio or text out.
Cloud notes	Next Moving the cloud model to a service covered by a Business Associate Agreement before real patient data is sent.
HIPAA program	Next Risk analysis, policies, training and BAAs with clinics and vendors. HIPAA has no official certification; this is how compliance is shown.
SOC 2 Type II	Planned Independent audit of security controls over an observation period.
Billing integration	Next Direct submission to a clinic's billing system or clearinghouse API.

Pilot program

Pilot clinics get discounted pricing in exchange for feedback. Their results set the time and revenue figures we publish.

Availability

Android now. iOS, web checkout, and links for YouTube, Instagram, LinkedIn and the app stores will be added at launch.